



AAROGYA SRI

AI-Powered Multilingual Health Assistant

Trusted healthcare guidance — for every language, every home.

**COURSE: PRODUCT MANAGEMENT WITH GENAI
AND AGENTIC AI**

By:

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PROJECT PLAN

1. Project Objective

The objective of the Aarogya Sri project is to provide accessible, reliable, and affordable healthcare information to non-English-speaking and underserved communities using AI-based voice and text agents. The system aims to reduce dependency on unreliable online sources, minimize healthcare costs through awareness of government schemes, and deliver health guidance in local languages for users with basic mobile devices.

2. Problem Scope Definition

Providing verified, multilingual health information for basic medical queries.

3. In-Scope Feature List

- Multilingual voice and text interaction
- Symptom-based general health guidance
- Preventive health tips and daily wellness advice
- Government health scheme awareness
- Cost-effective healthcare guidance
- Simple and intuitive user interface
- Works on low internet connectivity

4. Out-of-Scope Feature List

- Disease diagnosis and clinical decision-making
- Emergency response handling
- Doctor-to-patient chat or video calls
- Medicine recommendation or prescription
- Integration with hospital systems

5. Success Criteria Definition

- More than 40% of onboarded users receive accurate responses to healthcare queries
- Users successfully receive health information in their preferred local language
- At least 20% of users report reduced healthcare expenses through subsidies or affordable care options
- High user engagement and repeated usage of the platform
- Positive feedback regarding usability and trustworthiness

6. Project Timeline

Phase	Duration
Requirement Analysis	September
System Design	October
AI Model Selection & Backend Setup	October
Frontend Development	November
Voice & Language Integration	November
Testing & Validation	December
Deployment & Documentation	January

7. Milestone List

Milestone	Description
M1	Problem definition and user research completed
M2	Feature list and system design finalized
M3	Backend and AI model integrated
M4	Frontend UI developed
M5	Multilingual voice support implemented
M6	Testing and feedback incorporated
M7	Final project submission

8. Role and Responsibility Matrix

- **Vigneshwar – Frontend & Backend Development**
Responsible for building the user interface, implementing backend logic, integrating APIs, and ensuring end-to-end system functionality.
- **Mayurika – Testing & Quality Assurance**
Responsible for testing application features, identifying bugs, validating workflows, and ensuring product stability and reliability.
- **Anusriraje – Requirement Analysis & PRD**
Responsible for product planning, PRD creation, defining requirements, coordinating components, and preparing final documentation and presentation.
- **Ananya – User & Market Research**
Responsible for conducting user research, problem validation, and competitive analysis.

- **Vaibhav – UI/UX Design**
Responsible for designing wireframes, user flows, and ensuring a user-friendly and intuitive product experience.

9. Tool and Tech Stack List

- **Frontend & Low-Code Platform:**
Lovable – Used for rapid UI development and building the product interface using a low-code approach.
- **Backend & Database:**
Supabase (PostgreSQL) – Provides backend services including authentication, APIs, and secure data storage.
- **AI & NLP:**
LLM APIs – Power conversational intelligence and decision logic.
Speech-to-Text APIs – Convert user voice input into text.
ElevenLabs – Enables Text-to-Speech (TTS) for voice-based responses.
- **Workflow & Automation:**
Micro – Used for workflow orchestration and lightweight automation between services.
- **UI/UX Design:**
Figma – Used for creating wireframes, user flows, and high-fidelity design prototypes.

10. Demo Plan

1. User selects preferred language
2. User asks a health-related query via voice or text
3. AI processes and validates the query
4. Response is delivered in local language
5. Cost-saving options and government schemes are displayed

11. Final Delivery Checklist

- ✓ Functional prototype
- ✓ Multilingual support enabled
- ✓ AI voice and text interaction
- ✓ Project report
- ✓ Presentation slides
- ✓ Demo video or live demo
- ✓ Testing and feedback report

VALUE PROPOSITION

A. CUSTOMER PROFILE

1. Customer Jobs

Customers want to:

- Investigate payment disputes quickly and accurately
- Collect and submit correct evidence for chargebacks
- Reduce manual errors in dispute workflows
- Track dispute status across multiple payment providers
- Meet compliance and audit requirements
- Collaborate efficiently with risk, operations, and support teams

2. Customer Pains

Customers struggle with:

- Time-consuming manual evidence collection
- Missing or incomplete documents leading to case losses
- Confusion caused by multiple dashboards and tools
- Lack of clarity on dispute timelines and deadlines
- Difficulty identifying root causes of disputes
- High stress due to large dispute volumes
- Financial losses caused by operational mistakes

3. Customer Gains

Customers want:

- Faster dispute resolution cycles
- Higher dispute win-rate percentages
- Clear, structured workflows
- Automated evidence generation
- Real-time status updates
- Improved productivity and efficiency
- A single system to manage all disputes

B. VALUE MAP

4. Products & Services

The product offers:

- **End-to-end dispute and chargeback management platform**
- **Automated evidence builder**
- **Centralized dispute dashboard**
- **Workflow automation for case handling**
- **SLA alerts and deadline reminders**
- **Compliance-ready audit trails**
- **Analytics and reporting on dispute outcomes**

5. Pain Relievers

The product helps by:

- **Automating workflows to reduce manual errors**
- **Providing a unified platform to eliminate tool switching**
- **Offering real-time updates to prevent delays**
- **Displaying clear dispute timelines and next steps**
- **Using standardized evidence templates to avoid missing documents**
- **Sending alerts to prevent missed SLAs and deadlines**

6. Gain Creators

The product creates value by:

- **Increasing dispute win rates**
- **Reducing operational workload by 40–60%**
- **Ensuring regulatory and audit compliance**
- **Improving cross-team collaboration**
- **Providing analytics for data-driven decisions**
- **Enabling fast onboarding with minimal training**

1. What exact problem does your product solve?

Aarogya Sri addresses the lack of reliable, affordable, and local-language health information for non-English-speaking and underserved communities who struggle to navigate healthcare systems.

2. Who is the primary user of your product?

Low- to middle-income individuals and families in Tier 1, 2, and 3 cities using entry-level Android phones and speaking regional Indian languages.

3. What is the core value your product delivers?

Trusted, AI-powered health information and guidance delivered through voice and text in the user's preferred local language.

4. Why does this problem matter to the user?

Language barriers, misinformation, long hospital wait times, and high healthcare costs delay timely and informed health decisions.

5. How is your solution better than existing alternatives?

Unlike generic search engines, Aarogya Sri provides verified, localized, conversational health guidance tailored for underserved users.

6. What is your one-line value proposition?

Aarogya Sri provides trusted health guidance in local languages using AI, anytime and anywhere.

7. Why would a user choose your product today?

Instant access to reliable health information without hospital visits or language barriers.

8. What user pain does your product reduce the most?

Uncertainty and anxiety caused by lack of trustworthy and understandable health information.

9. What outcome does the user achieve using your product?

Clear understanding of health issues, preventive care guidance, and access to affordable healthcare options.

10. What assumption are you making about user value?

Users prefer trustworthy, local-language health guidance over generic or English-only information.

11. How did you validate that this value matters?

Through observed digital health behavior trends and feedback from underserved communities.

12. What happens if this value proposition fails?

Users continue relying on misinformation or delay healthcare decisions, increasing health risks.

13. Which feature directly supports your value proposition?

Multilingual voice- and text-based AI health assistant with verified content.

14. What trade-offs did you make to focus on this value?

Focused on information access and guidance instead of complex diagnostics in early phases.

15. How will you measure whether users get this value?

User engagement, accuracy of responses, repeat usage, and uptake of affordable healthcare services.

MVP DESIGN

User Flow (AI Agent)

- User enters health concern
- AI agent asks clarifying questions
- Provides verified guidance
- Suggests next action (doctor/helpline)

Information Architecture

- Home
- Ask AI
- Health Topics
- Emergency Help
- Profile & Language Settings

Wireframes

- Low-fidelity: Basic layout
- Mid-fidelity: Clear content hierarchy
- High-fidelity (optional): Final UI visuals
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VIABILITY CHECK

1. Market Demand Assumption

Assumption: There is strong unmet demand for trustworthy, multilingual health information in India.

Rationale:

- India has 600+ million non-English speakers who struggle to access credible digital health information in their preferred language.

- Existing health-seeking behavior relies heavily on WhatsApp forwards, local hearsay, and unverified online sources, increasing misinformation risk.
- The product directly addresses high-frequency, high-severity pain points such as language barriers, misinformation, and delayed care-seeking.

Conclusion: Demand is large, persistent, and structural, not trend-based.

2. Willingness-to-Pay (WTP) Assumption

Assumption: Users are willing to pay a small amount for trusted health guidance that reduces uncertainty and anxiety.

Rationale:

- Health is a high-stakes, high-trust category, where users are more open to paying if credibility is ensured.
- Target users (farmers, daily wage-workers, low & mid income families) may not afford expensive consultations but can pay for low-cost, value-added services.
- Willingness is higher for human-backed services such as nurse connect and personalized guidance.

Conclusion: WTP exists, but pricing must remain affordable and value-driven.

3. Revenue Model Selection

Selected Models (Hybrid):

- Freemium model: Free access to basic AI health Q&A
- Premium subscription: Advanced features, tracking, and deeper explanations
- Paid nurse support: On-demand human assistance

Rationale:

- Freemium ensures mass adoption and trust-building.
- Premium layers monetize users with higher engagement and needs.
- Nurse support enables transactional revenue without full clinical liability.

Conclusion: Revenue model balances scale, trust, and sustainability.

4. Pricing Assumption

Assumption: Users will accept low-cost & need-based pricing rather than monthly subscription fees.

Indicative Pricing Logic:

- Entry premium: ₹49–₹99/month - pay for months when used. Monthly 2 free questions can be given.
- Nurse connect: Pay-per-use

Rationale:

- Comparable to mobile recharges or OTT subscriptions.
- Low friction for rural and semi-urban users.

- Encourages long-term retention rather than one-off usage.

Conclusion: Pricing is context-sensitive and adoption-friendly.

5. Cost Estimate

Major Cost Heads:

- AI inference and model usage
- Cloud infrastructure and hosting
- Engineering and product maintenance
- Human nurse support (variable cost)

Rationale:

- AI and cloud costs scale efficiently with usage.
- No physical infrastructure required.
- Human support is optional and revenue-linked, keeping margins controllable.

Conclusion: Cost structure is predictable and scalable.

6. Resource Availability Check

Key Resources Required:

- AI models (language + health information layers)
- Curated and verified health content
- Partner nurse network

Rationale:

- All resources are readily available in the current ecosystem.
- No dependency on rare hardware, licenses, or regulatory approvals at initial stage.

Conclusion: Resource availability is high and non-restrictive.

7. Operational Feasibility Check

Assessment: Operationally feasible at scale.

Why:

- Fully digital, app-based delivery
- Multilingual AI handles most queries autonomously
- Clear scope boundaries (no diagnosis, no prescriptions) reduce legal and clinical risk
- Escalation to nurses only when required

Conclusion: Operations are lean, scalable, and low-risk.

8. Competitive Viability Assessment

Competitive Landscape:

- Existing platforms focus on English-speaking, urban users.

- Many lack local-language depth and verified information.
- Others either provide generic content or attempt risky diagnosis.

Swasth Saathi AI Advantage:

- Language-first design
- Verified, safety-bounded AI
- Trust positioning rather than speed-only positioning

Conclusion: Strong differentiation in a weakly served niche.

Overall Viability Verdict

Swasth Saathi AI is commercially, operationally, and competitively viable, with:

- Large unmet demand
- Realistic monetization paths
- Controlled costs
- Clear differentiation
- High scalability potential

USES OF AI

1. AI Use Cases in the Product

- Multilingual Health Q&A
- Personalized Daily Health Tips
- Wearable Data Interpretation (MVP+ feature)
- Smart Triage & Escalation

2. GenAI & Agentic AI Behaviour

- Auto-simplifying medical information
- Asking clarifying questions
- Sending daily reminders
- Maintaining context across sessions

3. AI Agent Responsibilities

- Multilingual understanding
- Verified, non-diagnostic guidance
- Personalization
- Safety enforcement

4. AI Agent Triggers

User Triggers: Health queries, translations, voice input

System Triggers: Daily tips, wearable alerts, inactivity re-engagement

5. AI Boundaries (Safety Rules)

Not Allowed: Diagnosis, prescriptions, emergency advice

Allowed: Education, risk awareness, preventive guidance

6. Failure Handling

- Low confidence responses
- High-risk escalation
- Clarifying prompts
- Safe fallback messages

7. AI Success Metrics

- $\geq 85\%$ medical accuracy
- $< 5\%$ hallucination rate
- $\geq 90\%$ red-flag detection accuracy

METRICS & MEASUREMENT

1. User & Customer Metrics

- DAU / MAU
- Activation Rate
- CSAT / NPS
- Query Completion Rate

2. Business & Revenue Metrics

- Free $\rightarrow$ Premium Conversion Rate
- Cost per User (CPU)
- Lifetime Value (LTV)
- Nurse support operating margin

3. Product Usage & Engagement Metrics

- Average queries per user
- Feature usage breakdown
- Retention (Day 7 / Day 30)

4. AI Performance Metrics

- **Medical Accuracy Score**
- **Hallucination Rate**
- **Red-Flag Detection Accuracy**
- **Language Accuracy & Clarity**

5. Growth Metrics

- **Referral rate**
- **Region-wise adoption**
- **User acquisition cost**

6. Quality, Trust & Reliability Metrics

- **App uptime**
- **Crash rate**
- **Response time**
- **Safety violations**

7. AI / Data / Automation Metrics

- **% queries resolved without nurse intervention**
- **Automation coverage**
- **Data completeness rate**

8. Combined Quantitative & Qualitative Metrics

Quantitative: DAU, accuracy, retention, conversion

Qualitative: Trust, clarity, ease of use, nurse feedback